



Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Mental Health (MH) Wards or CMHS Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPG)	Inpatient wards and Community Mental Health Services (CMHS)	Medical, Nursing, Allied Health

Policy statement

This policy outlines the requirements for the medical review of consumers managed within inpatient and community mental health clinic settings in order to promote comprehensive multi-disciplinary consumer care.

Patients and consumers are terms used interchangeably; this Policy will refer to “consumers”.

Links within document

- [Appendix I: Minimum medical review frequency in Mental Health inpatient ward /unit \(excluding MHEC\)](#)
- [Appendix II: TCU minimum medical review frequency in Mental Health inpatient ward /unit](#)
- [Appendix III: All Community settings minimum medical review frequency in Mental Health](#)

Procedures

Medical reviews involve an assessment of a consumer’s mental state and risk profile, focussing on risk of harm to self and others. This will inform the consumer’s treatment, management, and care, in order to optimise consumer outcomes by an allocated medical practitioner (Registrar, supervised Medical Officer [MO] or Psychiatrist) or medical team.

- A. On admission and on a regular basis:** The Medical Practitioner (MP) will complete an assessment of all consumers and document this on the Mental Health Assessment Form and other documentation. Refer to the **RPG Comprehensive Care SOP**, **Community Mental Health Services Triage to Discharge SOP** or **WA Department of Health Statewide Standardised Clinical Documentation (SSCD) for Mental Health Services**

Exclusion: Mental Health Emergency Centre (MHEC). Consumers receive a medical review in the Emergency Department prior to transfer to MHEC.

Procedures cont'd

B.

- I. **During admission** the frequency of medical reviews is guided by the timepoint in the delivery of care (e.g., on admission to the service, at the point of transfer or discharge) and acuity.
- II. Consumer identified as at risk of injury to self or others or from others.
- III. Major change identified in consumer's mental state.
- IV. Consumer experiencing physical and/or mental health clinical deterioration.
- V. Change in consumer's mental health legal status (Mental Health Act [MHA] 2014).
- VI. In response to an Emergency Response procedure (e.g., Code Black or Blue). (See **EMHS Emergency and Disaster Management Policy**).
- VII. Consumer identified as in need of seclusion (MP must be notified of seclusion event and review consumer post 2 hours from commencement of seclusion to determine if seclusion will be extended or revoked or continued and within 6 hours from release of seclusion/restraint (inpatient wards). Refer to **Minimisation and Safe Management of Restraint and Seclusion (for Mental Health Areas) Clinical Practice Standard (CPS)**.

C. Determined by Consultant Psychiatrist or Senior MP for medical review to be less frequent than the minimum 20 working days for community settings.

D. Before transfer of care or discharge: in order to determine an accurate status of the consumer's physical, mental state and capacity and current risks before handover to another treating team or discharge to a primary care setting.

Medical review frequency

A guide to the frequency of medical review is outlined in Appendices [I](#), [II](#), and [III](#).

When completing medical reviews, clinicians are to:

- Involve the consumer's personal support persons (PSPs) or guardians in any decision making, where possible
- Use the principles of recovery oriented mental health practice in accordance with the [Schedule 1 - Charter of Mental Health Care Principles](#).

Given the short stay nature of **MHEC**, consumers admitted to MHEC will have a face to face medical review daily. Refer to **MHEC Referral, Admission, Discharge and Transfer Policy** for other review requirements.

Given the Bidi Wungen Kaat Centre (Transitional Care Unit [TCU]) is a rehabilitation service that is sub-acute in nature, newly arrived residents are required to have a medical review by the Registrar on the same day and to be reviewed by the Psychiatrist within 72 hours. Note that medical cover is not available on weekends and public holidays. The minimum frequency of ongoing medical reviews is:

- Prevention and Recovery Unit: weekly basis
- Rehabilitation and Recovery Unit: four weekly basis

Refer to [Appendix II: TCU minimum medical review frequency in Mental Health inpatient ward /unit](#).

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DMR Amendment:	Clinical Practice Committee Chair OOS 20/01/2025		

Procedures cont'd

Documentation

Inpatient

All MPs are to clearly document their medical assessments on admission in the integrated notes and note the completion of the below on the *Multidisciplinary Comprehensive Care Plan* (inpatient only) (blue section to be completed by the medical team), including:

- MHA status
- Leave status
- Level of observations
- Frequency of medical review
- Predicted Discharge Date
- Completion of *Risk Assessment Management Plan (RAMP)*
- If any changes, document on *Progressive Risk Assessment (PRA)* (except TCU), including risk of harm to self or others
- Communication with the consumer and/or PSPs or guardians (if possible) about the outcomes of the review and management plan.

As leaders of junior medical officers (JMOs) and the multi-disciplinary teams (MDTs), the lead, inpatient unit Consultant Psychiatrists are expected to provide assessments of consumers at key intervals and document these assessments in consumer's healthcare record (see Appendices [I](#) and [II](#)).

Medical staff to complete handover of information from medical review and document key information in Psychiatric On Line Information System (PSOLIS) and other documentation as per **Clinical Handover Policy**.

Community

Consultant Psychiatrists provide oversight of the management of consumers managed by the JMOs they supervise, during MDT meetings and on an ad hoc basis as required (see Appendix III). Refer to **Community Mental Health Triage to Discharge SOP**.

- Registrars and Medical Officers are to discuss all consumer discharges with the supervising Consultant as well as the MDT
 - For consumers of CMHS, provide regular, minimum 3 monthly written updates to the consumer's general practitioner (GP) and other involved agencies, following a medical review and/or change to the consumer's medication regime or treatment plan or when significant mental state changes occur
- Medical staff to complete handover information from medical review and document in patient's healthcare record and key information in PSOLIS and other documentation as per **Clinical Handover Policy**.

RPMG staff to use versions of the SSCD that are on WebPSOLIS. For documents that are not available on PSOLIS, staff must complete the SSCD documents in paper-based or [sanctioned writable PDF format](#). Refer to the **WA Department of Health Triage to Discharge Mental Health Framework for SSCD** for details of which SSCD documents are available on PSOLIS.

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Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Progressive Risk Assessment \(PRA\) SOP](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [RRAD in Mental State, Cognition and Behaviour SOP](#)
- [Escalation to Consultant Policy](#)
- [Special and Supportive Observations Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Comprehensive Care SOP](#)
- [Community Mental Health Triage to Discharge SOP](#)
- [Minimisation and Safe Management of Restraint and Seclusion \(for Mental Health Areas\) CPS](#)
- [MHEC Referral, Admission, Discharge and Transfer Policy](#)
- [Clinical Handover Policy](#)

EMHS Policy Hub

- [Care Coordination in Mental Health Services Policy](#)
- [Emergency and Disaster Management Policy](#)

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Related policies, practice standards, clinical guidelines cont'd

WA Department of Health Policy Hub

- [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
- [Triage to Discharge: Mental Health Framework for SSCD](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Mental Health Act \(MHA\) 2014](#)
- [Mental Health Act \(2014\) Charter of Mental Health Care Principles](#)

Related forms

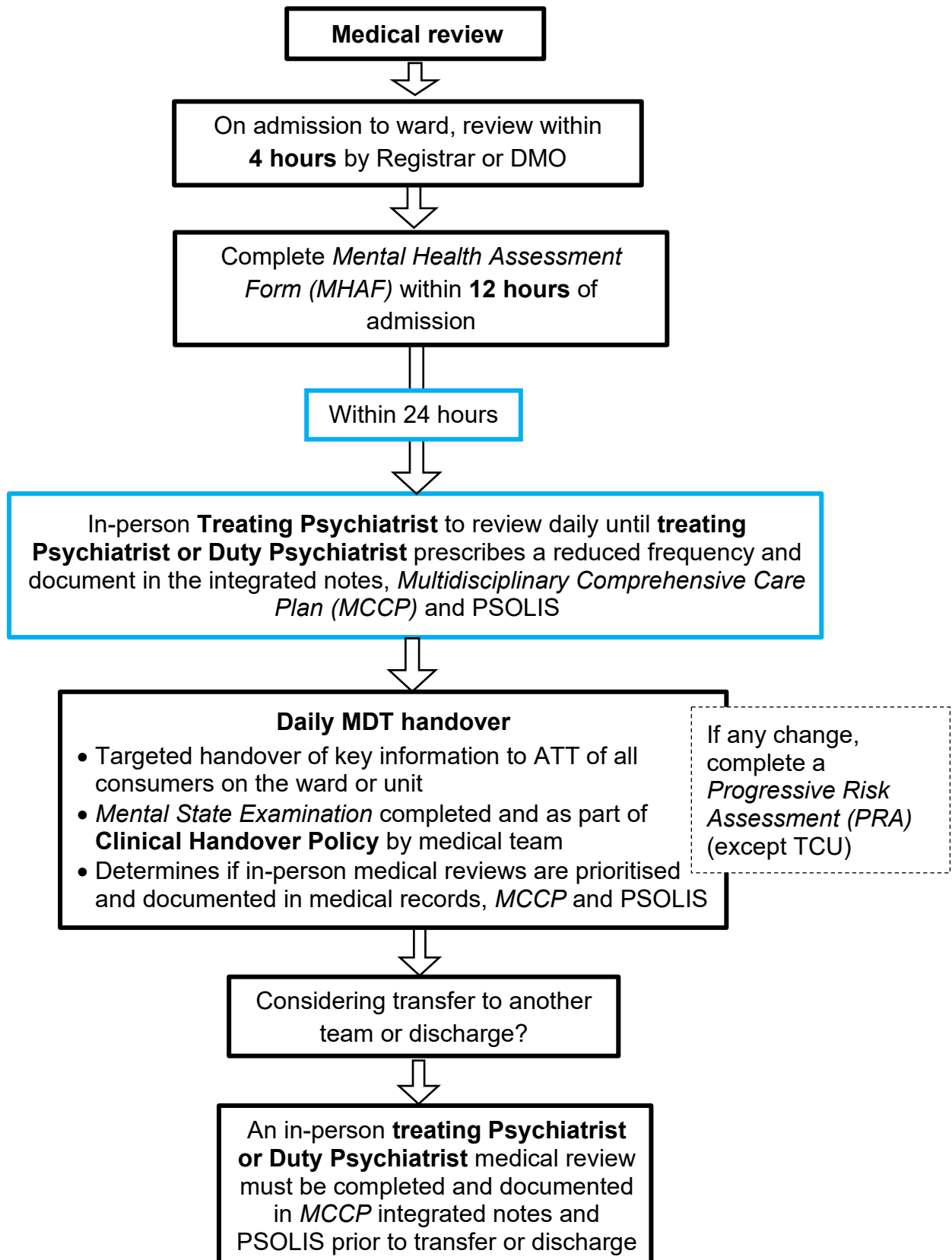
- *Multidisciplinary Comprehensive Care Plan*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021): Standard 2: Partnering with Consumers; Standard 5: Comprehensive Care; Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010): Standard 2: Safety; Standard 6: Consumers; Standard 10: Delivery of care

Appendix I: Minimum medical review frequency in Mental Health inpatient ward /unit (excluding MHEC)

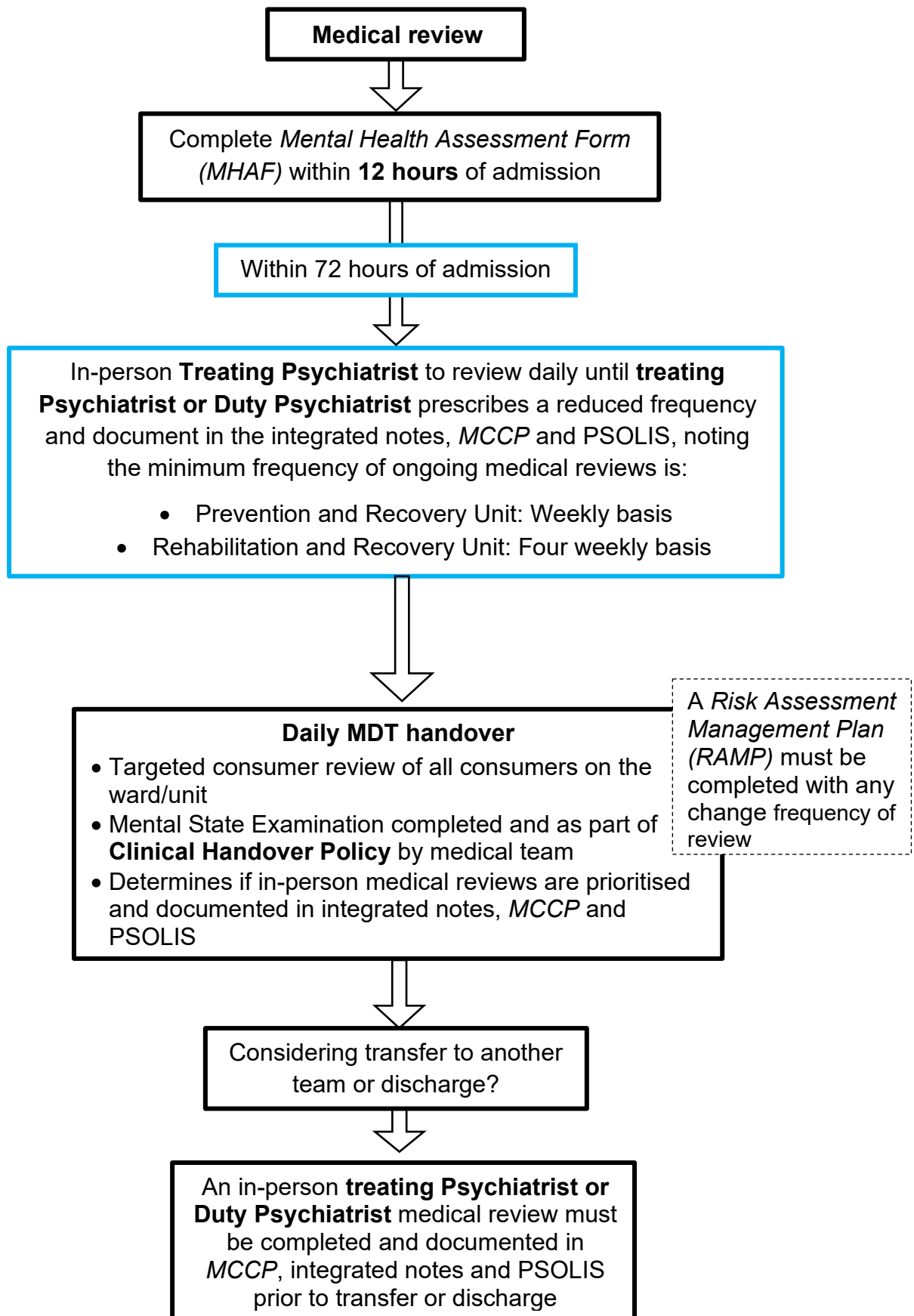


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Appendix II: TCU minimum medical review frequency in Mental Health inpatient ward /unit

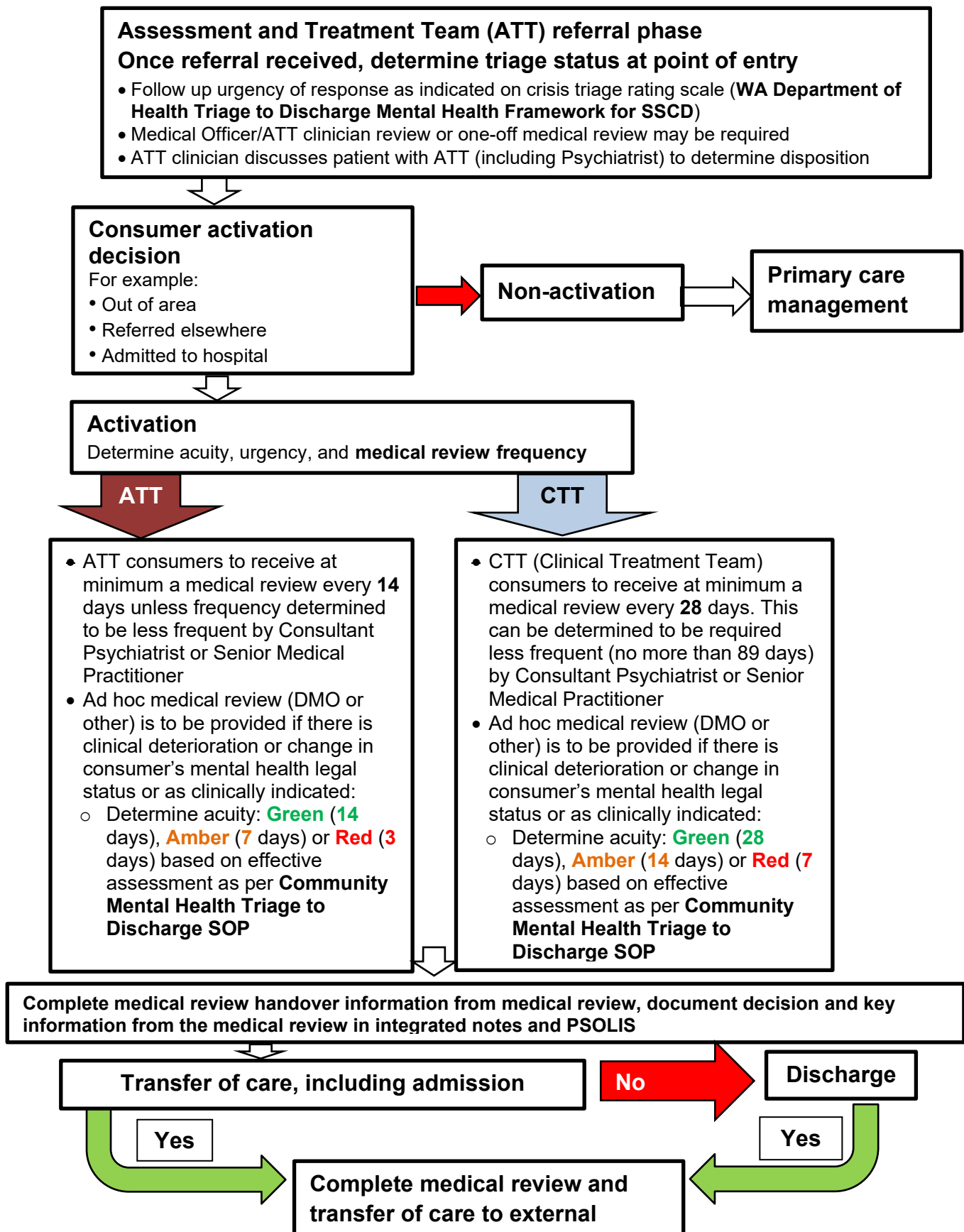


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Appendix III: All Community settings minimum medical review frequency in Mental Health



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